



SOUTH ASIAN JOURNAL OF
TRANSACTIONAL ANALYSIS

ESTD IN 2015 BY SAATA

VOLUME 12, NUMBER 2, SEPT 2026



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EDITORIAL SECTION

EDITOR'S NOTE

SAJTA is a journal for the community and by the community. The two people we want to speak to are the ones who make this journal possible: our authors and our readers.

To our authors, we express deep gratitude for sharing their labour of love with us and giving us a chance to showcase their work. To our readers, we have a sincere request – to spread the word about SAJTA. To let people know that they can find rich, beautifully written articles here.

Part of SAJTA's inception was to give South Asian practitioners a stage: to use their own voices and share what they know, how they practise, and what they see. Much of the theory we trained in was written elsewhere, for other rooms, with other families in mind. What happens to that theory in our consulting rooms, in our languages, with our clients and their obligations, is knowledge only we can produce. The papers in this issue do that work.

We find the process of writing itself quite magical – the way stray thoughts become ideas, ideas become words, and finally words become print – a journey along which the author acquires their voice and their own unique way of bringing out what incubated in their mind for long enough.

With AI entering this territory, we sometimes wonder if the story of writing will ever be the same. Will the writer's journey now invariably have AI as

company? Words that were once meticulously thought about and carefully chosen can now be effortlessly replaced by better and polished substitutes using AI.

We acknowledge that AI brings universality, accessibility, efficiency and an ability to equalise language deficits. But we also wonder - are we losing touch with the vulnerability that allows us to be seen the way we are? Are we trading our original voices for an AI-perfected version? And in that process, are we outsourcing not just our thinking but also our creativity, our style and the unique subjectivity of our own expression?

What we want here is open, honest, and respectful conversation about AI use within our community. When we look around us, it is plain that our community is full of skilled, talented practitioners with a great deal to say. So, bring us the case that changed your mind, the theory that did not fit, the question you cannot put down and trust your original voice. We at SAJTA value it immensely.

We believe that human writing has the capacity to be courageously imperfect, yet perfectly acceptable. So, bring out your pens and put on your writer's hat. Our editors will be with you every step of the way

Laxmi Sivaram and Nikita Bandale,

Co-managing Editors,

SAJTA

EXPERT PANEL

Priya Veeraraghavan

We acknowledge and thank the Expert Panel for their valuable contribution to the authors' work and for upholding SAJTA's commitment to reflective, high-quality Transactional Analysis literature.

ABOUT THIS ISSUE

Welcome to the September 2026 issue of SAJTA. As the peer-reviewed journal of the South Asian Association of Transactional Analysts (SAATA), SAJTA offers a welcoming space for practitioners from diverse fields to share their perspectives, emerging theories, practice-based insights and research. We value thoughtful academic and scholarly dialogue and seek to nurture the exchange of ideas and knowledge among professionals working in Transactional Analysis (TA) and allied fields.

Behind every issue of SAJTA is a team of editors who give their time and efforts generously. Writing takes shape through conversation, and our editors make the process genuinely rewarding. Our sincere thanks to the editorial board for the dedication and effort that made this edition possible.

It is with great pleasure that the SAJTA team welcomes Laxmi Sivaram as Managing Editor. Laxmi brings to the position a rare combination of skilful editing and an eye for detail that misses very little, held together by an honesty and warmth that everyone who has worked with her will recognise. These are precisely the qualities that SAJTA needs now; we are excited for her term and wish her every success!

We're thrilled to introduce our new editor, Sharanya Prakash. Sharanya is a practicing therapist and a partner at Psylent Mindlabs where she works closely with schools, colleges, universities, and corporates. She is a passionate Bharatanatyam dancer and a budding Kathak artist. Welcome Sharanya, your

work across your practice and the arts will bring valuable perspective to the journal.

It is also time to bid a very reluctant farewell to Nikita Bandale, who in her short term as Managing Editor helmed the release of SAJTA's milestone anniversary release. Her spectacular efforts towards streamlining the editorial process, exceptional teamwork and keeping the best interests of the journal at the heart of all her endeavours is inspiring. She truly made SAJTA a better place to stay.

This issue of SAJTA brings together three papers that hold TA in conversation with allied disciplines, spanning work with children and adults, clinical practice and professional ethics. Drawing on play therapy, trauma neuroscience and psychoanalytic thought, these papers place TA alongside other frames of reference. In each of them, the practitioner brings themselves fully into the room and shares their meaning-making. This vantage point is a gift to the reader. Together, they offer theoretical grounding, and an honest view of how this work unfolds in our own context.

About the Articles

[It Was Not A Wolf: A Play Therapy Case Study of Trauma, Symbolic Identity and Integration by Mina Dilip](#) is the case study of a 14-year-old boy with a rare presentation involving lycanthropic features. Working non-directively with art and play, the author attends to the fear beneath the *wolf*. Neuroscience

helps inform her understanding of how trauma is held in the body, while TA offers a frame for the script beliefs that made the adaptation necessary.

[“Am I Fit Enough To See This Client Today?” Situated Capacity, Ethics, and Repair Competencies by Samiksha Jain](#) distinguishes certified competence, from situated capacity which is the therapist’s fluctuating fitness to stay present as circumstances shift. Two vignettes present what can happen when a reduction goes unnamed and what can facilitate repair. The paper argues that ethical practice in our context rests not only on the individual practitioner but also on training, supervision and professional systems.

[Emerging Out of the Narcissistic Eclipse: Shining Light on Narcissistic Defences and Their Impact on the Therapeutic Relationship by Sarmishta Mani](#) revisits narcissism as an ordinary part of early development before it hardens into defence. Through two clinical vignettes, the author unpacks how the processes of idealisation and denigration work to manage unbearable pain.

Her aim is to humanise a presentation that typically evokes wariness, because the entitlement is easy to see, but the loneliness beneath it is difficult to connect with.

We hope you enjoy reading this issue and would love to hear your views, ideas, questions, or feedback. You could write to any author and / or us at journal@saata.org.

EDITORIAL BOARD

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THEME FOR MARCH 2027 ISSUE

“Open Issue”

(No Specific Theme)

Deadline for submissions to March 2027 issue: **November 30th, 2026**

SAJTA accepts submissions throughout the year and reviews manuscripts on an ongoing basis.

Email: journal@saata.org

Please follow the [Guidelines for Writing for SAJTA](#) and the SAJTA [AI Disclosure Policy](#) before you submit.

ARTICLES

IT WAS NOT A WOLF: A PLAY THERAPY CASE STUDY OF TRAUMA, SYMBOLIC IDENTITY AND INTEGRATION

MINA DILIP

Abstract

This paper presents a case study of a 14-year-old boy diagnosed with a rare delusional presentation involving lycanthropic features. The therapeutic work centred on building connection and understanding the emotional experiences that lay beneath it. Using a non-directive play therapy approach, the process supported emotional regulation, symbolic expression, and integration. The paper also incorporates the therapist's reflections through a Transactional Analysis (TA) lens.

Keywords: non-directive play, play therapy, trauma, symbolic play, transactional analysis, lycanthropy, emotional regulation, delusional disorder

Introduction

Children do not always come to therapy with words. Sometimes, they come with images, sensations, behaviours; or in Sai's case, a belief that he was a wolf.

In clinical practice, such presentations can easily draw attention toward diagnosis and symptom management. However, my experience as a play therapist has consistently shown that beneath the most complex symptoms is a child attempting to make sense of overwhelming experiences.

While neuroscience offered me an understanding of how trauma becomes embodied within the nervous system, TA provided me with a relational framework for understanding the meanings Sai and I made of our experiences.

Therapeutic Framework

This case is drawn from my clinical practice, with identifying details altered to ensure confidentiality. Sai attended weekly 60-minute sessions for a year. The therapeutic approach was grounded in non-directive play therapy (Axline, 1969), integrating art, sand tray work, and sensory regulation techniques.

Parent sessions were included to support co-regulation beyond the therapy room, recognising that a child's nervous system is shaped within relational environments (Perry, 2006).

Given Sai's limited access to narrative memory and his tendency to communicate through behaviour and symbolic expression, non-directive play therapy provided developmentally appropriate avenues for emotional processing that verbal psychotherapy alone may not have afforded.

Case Presentation

Sai, a 14-year-old boy, was referred to me with a diagnosis of delusional disorder with lycanthropic features.

Clinical lycanthropy is a rare psychiatric phenomenon in which an individual believes they are transforming, or have transformed, into an animal, most commonly a wolf. It is generally understood as a form of delusional misidentification syndrome and has been documented across a range of psychiatric and neurological conditions, including psychosis, mood disorders, trauma-related presentations, neurological illness, and substance-related states (Blom, 2014).

A systematic review conducted by Blom (2014) identified only a small number of published cases of lycanthropy reported in medical literature between 1852 and 2020, highlighting both the rarity of the condition and the limited body of therapeutic research available.

Sai reported experiencing himself as a “wolf” during periods of distress. In the months leading up to the referral, Sai’s behaviour had become increasingly alarming. He would growl, bare his teeth, and make clawing gestures toward others, particularly during moments of heightened emotional arousal. These episodes were not merely internal experiences but were expressed behaviourally in ways that caused significant concern to his family and school.

On one occasion at school, he lunged at a classmate and poked the child’s eye, resulting in immediate disciplinary action and a week-long suspension. These incidents were perceived as both aggressive and unpredictable, leading to fear among peers and adults alike.

At home, his parents reported a further escalation. Sai began to eat food from bowls placed on the floor, imitating animal-like behaviour. He appeared, at times, disconnected from his usual sense of self, moving between states of withdrawal and sudden, intense agitation. These behaviours were deeply distressing for his parents, who described feeling frightened and helpless in the face of what they could not understand.

In response to the severity of his presentation, Sai was taken to a psychiatrist and placed under a brief psychiatric hold. He was started on anti-psychotic medication, which appeared to reduce the intensity of the symptoms during the acute phase. However, following discharge the behaviours gradually re-emerged, suggesting that pharmacological treatment alone did not address the underlying emotional distress.

When Sai first entered therapy, I was struck by the stark contrast within him. He presented as intellectually bright, articulate, and capable of abstract thought. Yet, emotionally and physiologically, he appeared dysregulated and hypervigilant. This dissonance is often observed in trauma presentations, where higher cortical functions develop alongside unresolved subcortical activation (Siegel, 2012).

His developmental history revealed a childhood marked by an unpredictable and physically punitive father. A significant incident occurred when Sai was eight years old, in which he was severely beaten. This resulted in a head injury after he fell against a metal gate and lost consciousness. Notably,

this memory was not initially accessible to Sai in narrative form, suggesting that it had been encoded as implicit, sensory memory rather than an integrated experience (Perry, 2006).

I understood Sai's behaviours as manifestations of a nervous system organised around threat, alongside a deeply held belief that "The world is dangerous and I am only safe when I am strong."

I experienced Sai's oscillation between vulnerability and aggression as both moving and confusing. In some moments, I encountered a frightened young person whose fear and insecurity evoked a strong need in me to protect him. In other moments, his hostility and defensiveness created distance between us, making it difficult to maintain contact with the underlying vulnerability. I found myself wondering whether these aggressive presentations functioned as a form of protection, guarding feelings that had become associated with danger or helplessness.

I also observed that becoming aware of my own internal responses made it easier for me to hold both his vulnerability and his defensiveness in mind simultaneously, rather than identifying him with either. I wanted to create a therapeutic space where previously protected feelings could gradually be acknowledged and explored.

Establishing Safety: Working with Suspicion and Testing

The early phase of therapy was characterised by suspicion and boundary-testing.

Sai often presented with apparent hostility; challenging my responses and, at times, attempting to provoke reactions. I noticed feelings of caution and uncertainty within myself, alongside a strong impulse to reassure and soothe him. I felt as though Sai was unconsciously trying to evoke my inner critic or my Critical Parent (Berne, 1961). He appeared to test whether I would become controlling, punitive, rejecting, or over-responsible, like his father, thereby replaying a familiar relational dynamic and confirming his existing script beliefs (Berne, 1961) about relationships and safety.

The Wolf as Script and Survival Strategy

I understood Sai's wolf identity as a creative adaptation. Faced with experiences of intense fear, helplessness, and powerlessness, he appeared to construct an identity organised around the qualities he most needed but did not have the capacity to muster: strength, protection, and control. In this sense, I felt that becoming the wolf may have served a protective function, allowing him to distance himself from intolerable feelings of vulnerability while simultaneously providing a sense of power and safety.

It also probably was his survival strategy - if others were experienced as threatening, then remaining vigilant and aggressive in defence made sense. If

vulnerability was associated with danger, then aggression and emotional distancing offered protection. It appeared externally as bizarre or frightening behaviour, but I saw it, at least in part, as a coherent survival strategy developed within the context of repeated experiences of danger and powerlessness.

Regulation Before Trauma Processing

Before engaging with traumatic material, Sai needed to experience safety within his body.

In simple terms, when the body begins to feel safer, the thinking brain can come back online, allowing the child to reflect rather than react (Siegel, 2012). I therefore focused on strengthening the functioning of the Adult ego state through grounding, diaphragmatic breathing, and orienting exercises, enabling more reflective and less reactive responses, thus more regulation.

As physiological arousal reduced, Sai appeared increasingly able to observe and reflect upon his internal experiences rather than becoming overwhelmed by them. This created opportunities for greater differentiation between past and present, increased awareness of feelings and triggers, and a growing capacity to respond to experiences with choice rather than through automatic script-driven reactions.

The development of Adult ego state became particularly important as the therapeutic work progressed from regulation towards exploration and integration.

Non-directive play became another avenue for facilitating regulation, where the therapeutic process remained largely child-led, with Sai choosing the materials, themes, and forms of expression that felt accessible to him at different stages of therapy (Axline, 1969). Rather than directing the content of the sessions, my role was to provide a psychologically safe environment with a range of expressive and sensory materials through which symbolic communication and regulation could emerge organically. The selection of mediums, including art materials, masks, and sand tray elements, was therefore guided by Sai's own internal process and readiness.

In this stage of therapy, Sai worked with finger paints and masks (Figure 1).



Figure 1: The "Wolf" Mask

Sai independently selected and painted the mask during the session, and over the course of therapy identified it as the 'wolf mask' and spontaneously associating it with his internal experiences of fear, aggression, and loss of control.

Sand Tray and the Emergence of Memory

When Sai engaged in Sand Tray therapy, I was struck by the way these objects appeared to facilitate access to an experience that had previously remained outside conscious awareness. Sai explored the symbolic power of miniatures in the sand tray.

He shared that the snakes represented his fear (figure 2), the skull held his feelings of grief and loss, while the bird's nest was a symbol of safety (figure 3).



Figure 2. Snakes Symbolising Fear



Figure 3: Skull Representing Loss

In one pivotal session, he placed a metal gate and a bed within the tray. On seeing these elements, he experienced a sudden recollection of the childhood incident in which he had been beaten by his father, fallen against a metal gate, and lost consciousness (Figure 4).



Figure 4: The Bed and Gate that Brought Back an Implicit Memory

For me this was significant because it was not simply about the gate or bed themselves, but their appearance within a symbolic medium that allowed previously fragmented sensory and emotional material to become accessible. The sand tray appeared to enable Sai to reconnect with an experience that had long been held outside his awareness. Watching the tray take shape, I felt a subtle shift in the room, a sense that we were approaching something that had been waiting for a long time to be seen.

Integration: From Fear to Possibilities

As therapy progressed, Sai began to be able to distinguish between his identity and the emotional experience that had come to define it. Rather than experiencing himself as the wolf, he developed the capacity to recognise and name the fear that lay beneath it. This shift created space for new possibilities: fear could be experienced without becoming his identity; vulnerability could be acknowledged without being overwhelmed by it; and protection no longer required the abandonment of emotional authenticity. What had previously been expressed through behaviour, symbolism, and identification could now be brought into awareness and reflected upon.

His drawings became more organised, his emotions more regulated, and his physiological state calmer as evidenced by his steady breathing, relaxed posture and longer conversations during sessions. His parents reported improved sleep, appetite, and daily functioning, indicators of reduced chronic arousal (Perry, 2006).

He then recreated the same scene symbolically using the following elements (Figure 5):

- A fallen child
- A threatening figure
- A protective presence on a mound



Figure 5: The Fallen Child, the Threatening Figure and the Protective Presence

Although the meanings of these figures remained Sai's own, for me this sand tray seemed to hold multiple aspects of Sai's internal world simultaneously: vulnerability, threat, and the emerging possibility of protection. The image of the fallen child alongside the threatening figure gave me a sense of how afraid, helpless, and isolated he must have felt. For a moment, I felt shaken by the emotional weight of what seemed to be emerging. I also noticed in me, a strong impulse to comfort and protect him. I recognized how difficult

yet important it was for the therapeutic process that I remained present, attuned and responsive as I acknowledged the significance of what he was showing me.

Such moments of attuned witnessing are central to trauma integration. When overwhelming experiences are revisited within a safe relational context, the brain can begin to link emotional and sensory memory with narrative understanding (Siegel, 2012). The presence of a regulated other supports the nervous system in remaining within a window of tolerance (Porges, 2011). For Sai, this created the possibility of reflecting on experiences that had previously remained outside conscious awareness, allowing both traumatic memories and the script decisions associated with them to become available for exploration and re-evaluation.

The Therapeutic Relationship as Corrective Experience

Sai's father remained emotionally distant throughout the process. This made the therapeutic relationship a critical site for healing. I became intentional about offering consistency, predictability and reliability along with clear, safe boundaries.

By offering a steady and accepting presence and maintaining predictable boundaries when Sai expressed fear, anger, or vulnerability, I conveyed that difficult feelings could be experienced without rejection or punishment. This required me to stay with his experience without rushing to interpret, reassure,

or move away from the discomfort it evoked, while remaining aware of my own emotional responses with curiosity and compassion. I felt deeply moved by Sai's vulnerability, while also holding a sense of humility as I accompanied him through experiences that felt both fragile and profound.

Through the consistent experience of acceptance, emotional safety, and non-punitive responsiveness within the therapeutic relationship, he was offered an opportunity to challenge and revise his earlier script beliefs that protection depended upon intimidation, aggression, or emotional armour (Berne, 1961).

Outcome and Termination

Over the course of approximately 12 months of therapy, Sai appeared increasingly able to experiment with expressing vulnerability directly, rather than relying solely on the protective identity of the wolf.

He cultivated the ability to identify triggers and regulate himself more effectively. These changes were observed both within sessions and through reports from his parents regarding his functioning at home and school. Regular quarterly follow-ups over the subsequent twelve months following termination revealed that the improvements appeared stable and sustained.

The final session involved a sand tray that Sai described in ways that suggested an increased sense of safety, transition, and self-agency (Figure 6).



Figure 6: Integration: The Final Tray

Discussion

This case illustrates how play therapy facilitates change by addressing underlying neurobiological and relational processes rather than symptoms alone. While most documented cases of clinical lycanthropy emphasise psychiatric diagnosis and pharmacological intervention, this case highlights the potential role of relational and symbolic therapies in addressing the underlying trauma and dysregulation associated with such presentations.

Through this process, Transactional Analysis offered me a framework for understanding the meanings he attached to these experiences and the adaptations that emerged in response.

The therapeutic process involved more than symptom reduction. As regulation increased, Sai demonstrated growing Adult functioning, evidenced by his ability to reflect on and make sense of his internal experience (Berne,

1961). A central aspect of the work involved the communication of implicit permissions (Crossman, 1966): “You can feel,” and “You are safe.”

As a therapist, my role was to facilitate the movement from his survival-based identity toward integration, while anchoring the process in relational safety and attuned presence.

Conclusion

Sai did not need to be convinced that he was not a wolf. He needed a space where the fear hiding behind the wolf could be safely experienced. Although this is only one child's story, it highlights the importance of looking beyond behaviour to the needs that may lie underneath it. For Sai, healing emerged not through doing away with the wolf, but through creating sufficient safety for the underlying fear to be seen, understood, and integrated.

AI Disclosure: The author affirms that AI was used only for editorial assistance and that all content presented is original and takes full responsibility for the work.

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About the Author

Mina Dilip is a psychotherapist based in Chennai, India. With a Master's degree in Psychology from the University of Madras and international certifications in Play Therapy and Sand Tray Therapy, she works with both children and adults using non-directive, trauma-informed approaches. Mina serves as a well-being supervisor under a global Employee Assistance Program run by CiC, UK; and is the current President of the Chennai Counsellors Foundation. A mentor and supervisor to upcoming therapists, Mina is passionate about emotional healing, co-regulation, and creative expression. Mina can be reached at mina.dilip@gmail.com.

“AM I FIT ENOUGH TO SEE THIS CLIENT TODAY?” SITUATED CAPACITY, ETHICS, AND REPAIR COMPETENCIES

SAMIKSHA JAIN

Abstract

Psychotherapy training and certification primarily assess competence at a single point in a therapist's career, but they cannot assess the therapist's ongoing capacity to enact that competence ethically across changing life circumstances. This paper distinguishes Certified Competence from Situated Capacity: the therapist's fluctuating fitness to remain present, attuned, and contactful amid changing health, priorities, relational demands, financial pressures, and structural conditions. The article uses two clinical vignettes to illustrate named and unnamed reductions and repair competencies to restore the alliance. Situated in the Indian context, the paper argues that ethical practice depends not only on individual therapists but also on supervisory, training, and professional systems.

Keywords: *Situated Capacity, Certified Competence, Therapeutic Presence, Relational Psychotherapy, Ethics, Financial Strain, Rupture, Accountability, Humility, Repair Competencies, Transactional Analysis, Process Contract, Psychological Contract, Protection, Permission, Potency, Burnout*

Introduction

Through a relational approach, the paper examines the therapeutic alliance to define a therapist's situated capacity and distinguish it from certified competence. The author's reflections on situated capacity emerged not only from clinical work but also from living with a chronic health condition that required a daily assessment of functional capacity.

Over time, bodily cues became reliable indicators of when therapeutic presence (Rogers, 1975) might be compromised. This daily practice of assessing fitness to practice gradually shifted the author's attention from competence as a stable professional attribute to capacity as a fluctuating, embodied, and context-dependent ethical consideration.

Certified Competence and Situated Capacity

Psychotherapy training culminates in certification or licensure: a rigorous assessment of theoretical knowledge and clinical competence at one point in a therapist's career. This is Certified Competence. What certification cannot assess is how competence is held and develops across the rest of a therapist's career.

This paper proposes a second, related construct. Situated Capacity is the therapist's moment-to-moment ability to bring the physical, cognitive, emotional, relational, reflective, and ethical resources required to work effectively toward agreed therapeutic goals. It is shaped by shifting priorities,

health fluctuations, relational upheavals, financial pressure, and inadequate structural support, operating both within and outside the therapeutic setting.

Fluctuations in situated capacity may affect presence, attunement (Erskine, 1993), contact, clinical judgment, responsiveness, and repair. Unexamined, such stressors enter the room unnoticed by therapist and client alike.

While certified competence establishes a professional foundation, situated capacity must be actively maintained through ongoing self-examination, personal therapy, supervision, and training spaces. Johnson et al. (2013) note that competence is fluid and degrades over time, practitioners are poor judges of their own competence, and clinical work offers little external feedback to correct the gap.

All errors do not stem from reduced capacity. A theoretical blind spot, a persistent countertransference pattern, or gaps in training produce competence-based errors: relatively consistent across a therapist's work and appropriately addressed in supervision. This paper proposes a second category; capacity-based errors which arise when a therapist who ordinarily maintains the alliance well loses access, for a session or a period, to competence they genuinely possess. Being uncharacteristic or circumstantial, they escape a system oriented towards assessing stable competence. One way to identify the category of error is to ask whether it is characteristic of this therapist or a departure from their usual practice.

Once certification is passed, competence can fade as an active question, and few contexts offer a consistent mechanism for re-examining it. Competence is not an attribute settled at examination but a daily interplay between training and what is being carried from life.

The ITAA Code of Ethics (ITAA, 2025) grounds practice in five principles: Responsibility, Respect, Empowerment, Protection, and Commitment in relationships. Each of these rests on the therapist's situated capacity, meaning emotional availability alongside physical and mental condition. The article explores what happens to the principles of Responsibility and Protection when capacity is reduced and goes unnamed.

Relationally, three concepts flow sequentially. Novellino (2012) describes the process contract as attending to the "how" of the work, including an agreement for either party to address anything that may weaken the alliance. Cornell (2016) describes the psychological contract as motives and expectations operating at a hidden level between client and professional that can hinder therapeutic goals. Tudor (2016) expands on three conditions necessary for therapeutic change: Permission, Protection, and Potency.

This paper proposes that these form a sequence. When situated capacity diminishes and goes unnamed, the process contract is strained on the therapist's side, and the psychological contract may activate; the client may adapt by withdrawing, self-blaming, or silencing their need, and the therapist's capacity for permission, protection, and potency may be unintentionally

compromised. The presence Rogers (1975) describes can become difficult to sustain, and the conditions for rupture may follow. Unaddressed, this can manifest as lapses in the ethical principles the ITAA upholds (ITAA, 2025).

Naming Reduced Capacity

Naming has two forms. Naming to oneself is the reflective act on which everything else depends, and without it, ethical choice may not be available. Naming in the therapy room is a discernment, contextual rather than universal. The ethical task is not disclosure itself but recognizing the reduction and then judging whether saying it aloud strengthens or burdens the alliance.

Reduced capacity and rupture remain distinct. Naming reduced capacity is proactive, an acknowledgment that presence may be affected. Naming a rupture comes after that reduction has already touched the relationship.

The two vignettes that follow illustrate each in turn. Both vignettes come from the author's practice over fifteen years, offered as honest case reflections.

Clinical Illustration: A Therapist's Unnamed Grief

This example illustrates how good intentions may not prevent harm when reduced situated capacity goes unnamed.

A client had been attending therapy for approximately six months. He was ambivalent about the process, stating each week that he did not know why

he came, yet he continued to attend. One afternoon, the author arrived at his session directly from a hospital.

The author had been caring for an ill family member largely alone, managing mounting medical costs and an unhelpful treatment, and carrying the anticipatory grief of a loss she could sense.

On the way, she noticed dread, a racing heart, and gastrointestinal discomfort, reliable markers of emotional dysregulation after years of practicing with a chronic health condition. The attention ordinarily available for presence was already occupied by regulating her own state.

She had been crying and arrived at the session with red eyes and a nasal voice. Rather than naming her state or offering to postpone, the author held the session as if unaffected and said nothing of where she had been. The intent was protective: she did not want to burden the client with her personal grief.

The client, however, noticed what was visible. "Are you okay?" he asked. "Yes, I'm fine. How are you doing?" the author replied. The exchange lasted seconds, yet it became the critical decision point: the client had accurately perceived her reduced capacity and opened a space for it to be named, and the author's reply declined the invitation.

Author's Reflections

The author felt that cancelling risked confirming the client's belief that therapy was not worth pursuing. Naming her state risked burdening a man whose lifelong pattern was to care for distressed elders. Continuing while appearing composed seemed, at the time, to protect the client, relationship, and her own circumstances at once. The author also feared losing the client which she felt she could not afford due to financial pressure.

The hesitation to cancel was not only an individual ethical dilemma. In general, therapists invest substantially in training, supervision, and personal therapy while often practicing under financial constraint, and only 20.9% of Indian therapists report feeling financially secure (Bharathi, 2026).

Insights From Personal Therapy and Supervision

The weight she carried alone had driven the protective impulse, and the wish to keep her grief from touching the client overrode honest acknowledgment of reduced capacity. The process contract, the agreement to name what may weaken the alliance, was breached unconsciously.

The psychological contract activated: the author would appear composed, and the client would protect her from feeling worse by silencing his own needs. Permission and protection failed, and with them potency; the therapist could not act as an agent of change (Crossman, 1966). Protection and Responsibility (ITAA, 2025) were also compromised.

Rupture and Repair Competencies

This paper further proposes a register of repair competencies: Integrity, Accountability, and Humility. These do not switch on at rupture; they are present across the work. Like certified competence, they are built over time through personal therapy, supervision, and intentional practice. However, repair competencies are rarely assessed. Whether a therapist can reach them under pressure is a question of capacity and may be a moment of ethical assessment.

Clinical Illustration: The Client Who Called Out the Therapist

A client with a long history of caretaking a depressed family member came to the session exhausted by a friend's emotional demands. One of her stated therapeutic goals was to stop rescuing others at the cost of her own well-being. The author, on the other hand, had for some time been feeling powerless in a personal relationship. A difficult conversation moments before the session stirred that powerlessness again, and it surfaced as an unconscious attempt to recover authority through the expert's position.

She responded: "Your friend is depressed; you can be the bigger one." The client recoiled, eyes widened in shock: "How can you say that to me?" Her body registered the contractual breach before her mind could process it.

The author noticed the heat rising in her chest, the headache that had followed the earlier conversation, and the mismatch between what she had just said and the client's therapeutic goal.

Unlike the first vignette, her internal state remained available to observation, and the client's protest brought her attention back to the present moment before the author's misplaced advice could reorganize the relationship. She acknowledged the slip, apologized, and reoriented the intervention to the client's goals.

Repair competencies remained accessible in this case, even though situated capacity was reduced. Responsibility and Protection (ITAA, 2025) were upheld in the immediate accountability for the slip and in the decision to repair rather than defend. Where the slip had briefly withheld permission (Crossman, 1966), instructing the client to do the very rescuing she had come to stop, naming it restored permission, protection, and potency.

Discussion

Safran and Muran (1996) established rupture and repair as central to the alliance, and Shadbolt (2012) extends this within the TA tradition, framing failure not as pathology but as communication to be engaged with. This paper proposes that reduced situated capacity itself exists on a spectrum, and that where a therapist falls on it governs whether repair is

possible. In the first vignette, cumulative caregiving, anticipatory grief, financial pressure, and prolonged physiological dysregulation occupied much of the reflective space ordinarily available for presence.

In the second, capacity was reduced to a lesser degree because the triggering event had already been worked on in personal therapy and supervision, leaving reflective awareness accessible enough for the client's protest to be validated rather than defended against.

Repair competencies also remained available because sufficient capacity was intact to sustain contact in both directions (Erskine, 1993), inward with her own state, outward with the client.

Enabling Capacities

Access to repair competencies rests on a layer of enabling capacities: tolerance of one's own shame, the ability to witness that experience while remaining oriented to the client's goals, and an Adult capacity to mediate between archaic experiences and the present context.

Genuine repair requires more than accountability. Humility asks one to sit with having caused harm without moving to explain, contextualize, or resolve it for one's own relief. Shadbolt (2012) terms this the therapist's vulnerability, arguing that open acknowledgment of failure transforms rupture into relational learning. Erskine (2011) likewise identifies willingness to take responsibility for therapeutic errors as essential to psychotherapeutic presence.

These skills depend on the therapist's capacity to step back from her own instinct to protect herself, to observe the ego's desire to seem capable without being dominated by it. This is what enables self-reflection to be open and non-defensive, rather than a form of subtle self-protection. When this ability breaks down, shame becomes unbearable, and defensiveness takes over. When it remains intact, the relationship can become a space for learning.

Repair competencies work both remedially and preventively. Preventively, the same competencies could allow a therapist to name a diminished capacity rather than maintain a posture of unbroken competence, potentially sparing the client from filling the gap with self-blame. Remedially, repair lets power flow in both directions, the client through self-advocacy and the therapist through accountability and humility, which can make the relationship more egalitarian. Trauma and body-oriented traditions already treat repair as a clinical requirement rather than an optional virtue (Pearlman & Saakvitne, 1995).

The Shared Ethical Responsibility

Johnson et al. (2013) propose a competence constellation, a communitarian model in which competence is sustained by a network of colleagues rather than held individually. Examining situated capacity likewise cannot rest on the individual practitioner alone. The ecosystem described here applies the same idea to capacity, asking what each part of the community is responsible for.

Therapists bear immediate responsibility for noticing reduced capacity and naming it when appropriate. Qualified practitioners can sometimes be reluctant to seek supervision, which Eusden (2011) refers to as a "narcissistic difficulty in seeking a third" (p. 108). Supervisors are responsible for creating shame-free spaces where self-examination can happen, and for asking whether an error is characteristic or a departure.

Trainers are responsible for modeling what professional development actually looks like; when they normalize fluctuations in capacity, trainees can learn that honest self-examination is the work itself. Instead, when their frame of reference is shaped by systemic and cultural inheritance, it may actually reproduce harm rather than interrupt it (Abdulla, 2023).

Responsibility within this ecosystem is not equal but increases with influence: the qualities demonstrated by those who train, supervise, and lead set the standard for others to follow.

Credentialing and governing bodies are responsible for making ethical practice sustainable. Simionato et al. (2019) established burnout as an ethical concern as well, since economic insecurity, high caseloads, and absent institutional support predictably reduce situated capacity. Fee structures, certification frameworks, and ethical principles should support the practices those bodies prescribe rather than place that burden on individual practitioners. Requiring ethical practice without building the systems that support it is itself untenable.

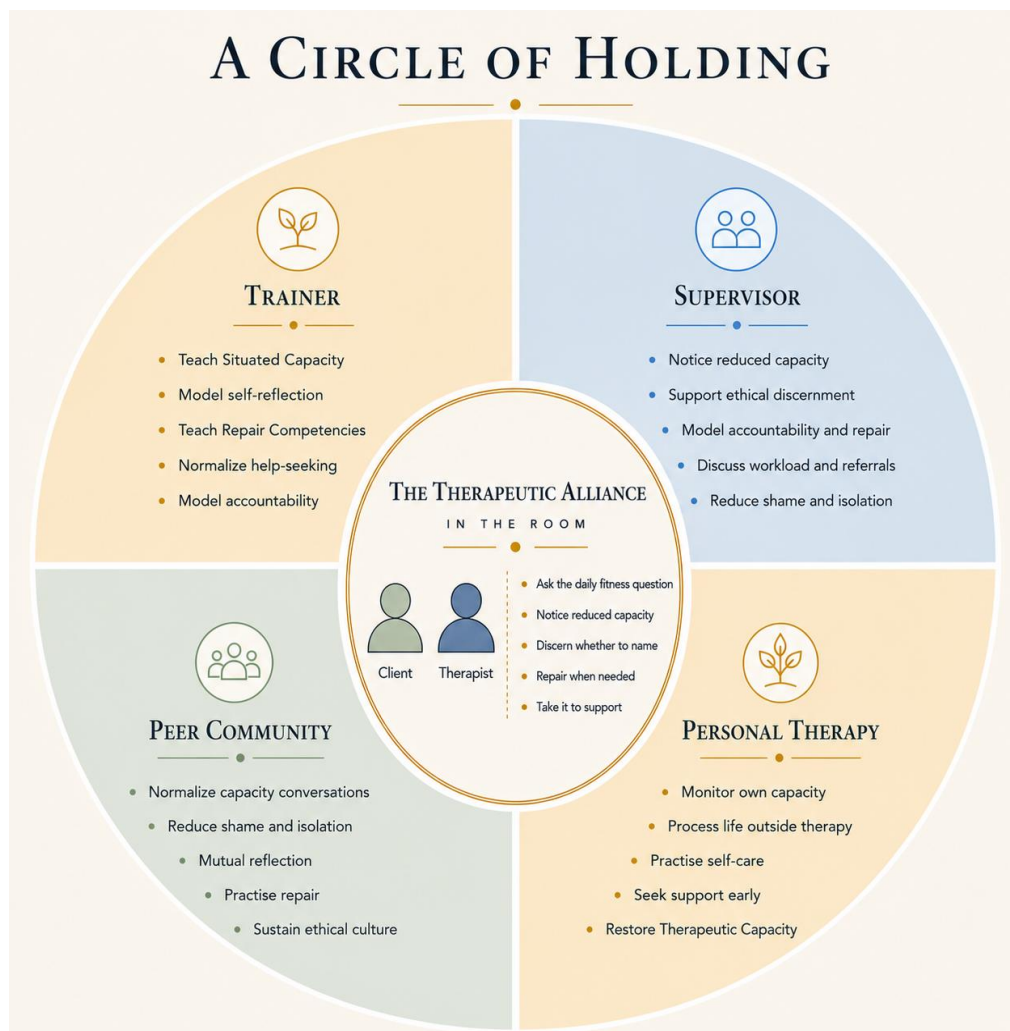


Figure 1. Circle of Holding

Conclusion

Certification grants competence. Life keeps testing capacity. The ethical question, “Am I fit enough to see this client today?”, this paper poses is whether one continues to ask, honestly and repeatedly, how circumstances shape one’s capacity to be present, attuned, and contactful in the session. That question belongs to the individual therapist and to the system at once. The therapist carries the daily work of noticing and naming, often amid very real personal and

structural strain. The system is responsible for creating the conditions that make such honesty possible rather than costly. Neither can hold ethical practice alone.

AI Disclosure: The author affirms that AI was used only for editorial assistance and that all content presented is original and takes full responsibility for the work.

Acknowledgements

The author wishes to acknowledge Haseena Abdulla (TSTA-P), whose supervision and scholarly work have informed her practice and the development of this paper.

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The construct of situated capacity emerged from her experience, both as a therapy client and as a practicing professional, and from more than two decades of working alongside a chronic health condition that produces fluctuating capacity. Her studies of Advaita Vedanta and the Bhagavad Gita continue to inform her reflections on ethics, responsibility, and therapeutic presence. This paper reflects the interplay of those professional and personal realities. Currently, she is working on the influence of structural conditions and Indian culture on the therapist's presence in a forthcoming companion paper. The author can be reached at samikshajain67@gmail.com.

EMERGING OUT OF THE NARCISSISTIC ECLIPSE: SHINING LIGHT ON NARCISSISTIC DEFENCES AND THEIR IMPACT ON THE THERAPEUTIC RELATIONSHIP

SARMISHTA MANI

Abstract

Our earliest encounter with narcissism takes place in childhood and is an inevitable part of the development of our personality. In healthy development, we learn to find a balance between our self-indulgent needs and our need for connection and cooperation with other people. When this balance has not been achieved, we are likely to cope with disturbances through narcissistic defences that protect us from anxiety and pain but come at the cost of loss of contact with our self, the world and with reality. The consequences are often disruptive and include dissatisfaction and abrupt endings in relationships, hurt or harm to the other person with little realisation of one's loss, further isolation - an existence in which one's vitality for connections and life could feel compromised. This paper is an attempt to present a more sensitive understanding of the person behind the experience. The author brings together theories from psychoanalytic literature and a few instances from her practice to weave a tapestry that includes imagination and lived experience.

Keywords: *Narcissism, Narcissistic Defences, Idealisation, Denigration, Splitting, Shame, Humiliation, Psychic Retreat, Psychoanalysis*

Introduction

The subject of narcissism conjures an image of a self-absorbed person who derives little meaning from relationships with other people, brings about destruction to people who might show care, and occupies a world in which a loveless and isolated existence is kept out of awareness by immersing oneself in love with an idealised version of the self. These descriptions are widespread, evoking feelings ranging from wariness to distrust, to even disgust. In this paper, I present some reflections that humanise the phenomenon, by bringing together theories and lived experiences in the therapy room with my patients.

Narcissism as a part of Normal Development

Freud (1914) defined narcissism as:

“The libido that has been withdrawn from the external world has been directed to the ego and thus gives rise to an attitude which may be called narcissism.”
(p.75)

He regarded the tendency towards narcissism in childhood as a vital and necessary stage between auto-erotism (an early state of libido) and object-cathexis, which is one's love and interest in another. By libido, I am referring to one's innate drive to creatively foster life. I am reminded of an incident when my son was 3 years old. I had cut a papaya into two halves. My son, his father and I were sharing one half. When it was finished, my son said “ma and pa's papaya is over, now I'll eat mine. This tendency of a child to place itself at the centre of its

world reveals an age-appropriate grandiose sense of self and is also a core feature of narcissism in adults (Hargaden and Sills, 2002).

Narcissistic Defences

From our earliest days our defences or means of self-preservation and protection are resources we build and tap into, as we navigate our various relationships. Freud considered it normal to carry a reasonable propensity for narcissism into adulthood. As a child grows, in healthy development the ego-libido (love towards the self) is balanced adequately with object-libido (loving and including the other).

In other words, with the early indulgence in narcissism adequately balanced by the child's need to connect and engage with the world, we may say that it is developing into a healthy individual who can manage caring for the self with care for others. Narcissistic functioning as described below indicates a failure in achieving this balance.

Popular representations of narcissism include lacking in empathy, being self-centred, full of selfishness and such, and there is indeed damage, whether as hurt or trauma, inflicted upon the other who has been blocked out, misused or abused. This is particularly evident in groups, especially those pitted against one another, where each side rigidly views itself as superior and denigrates the other.

In order to facilitate a shift from an existence with a shallow sense of self to a life of meaningful relationships with deepening connections that nourish the individual, their immediate circles and the larger humanity, I believe a sensitive understanding of the person behind the narcissistic presentation is vital. The sense of entitlement of the person constricted by narcissistic defences is easy to identify, but the suffering that follows a fragile self-esteem and loneliness is difficult to empathise with.

Mental Image and Self-Image

“A person acts and feels, not according to what things are really like, but according to his mental image of what they are like.” (Berne, 1971, p.45).

Berne held that the closer these images were to reality, the greater was one's chance for happiness in life. For a person who has been unable to establish a balance between ego-libido and object-libido and instead copes by resorting to narcissistic defences, the mental images are likely to distort reality. The degree of narcissism impacts the level of satisfaction one experiences from contact with oneself and with others.

I now share a couple of experiences with my patients.

The Therapist is the Threat

Miya was a 40-year-old woman. Both her parents were successful businesspeople. She was born from an unexpected pregnancy, 15 years after her brother. She believed that her parents adored him, while she would constantly

battle a sense of being subsumed by them. Miya ran a successful business herself but disliked the fact that her parents had provided her the capital.

The facilitating environment (made up of significant others) is fundamental to the survival and development of the baby and can be inferred in the idea that there is no infant without the mother/other (Winnicott, 1960). This includes good-enough fulfilment of the need of the baby to feel central in the lives of its caregivers and being supported by the environment to outgrow this need; in other words, achieving the balance between care for self and care for others.

A baby that has experienced repeated emotional neglect develops a personality that anticipates chaos from re-experiencing trauma and erects defences for protection. Miya built an armour of narcissistic defences such as splitting, projective identification, primitive idealisation and denigration, that protected her from an acute sense of worthlessness.

In our sessions, Miya would be polite and articulate, but I sensed the distance in her engagement with me.

Around a year into therapy, Miya had some business meetings and informed me that she would miss a couple of sessions. When she missed the third session, I sent her a message about the missed session. Miya came to the following session and informed me that she was stopping therapy since it had not helped her.

Recovering from my shock, I explored her sudden decision with her. I learnt that Miya had become very anxious when she realised that she had forgotten the

session. And the only reason she could attribute to her behaviour was that our work must have been ineffective. In denigrating our work (and me), she could evoke her armour of narcissistic defences to protect herself from collapsing into caustic self-blame and the accompanying crippling shame.

“To live in a world of idealisation is to bask in the radiant light of the idealised object” (Bollas, 2021, p. 12)

Describing manifestations of narcissism in a patient, Bollas cautions the therapist about the dangers to the patient of fragmentation or a breakdown into a scattered state, when the therapist is no longer experienced as passive and supportive; i.e., the therapist is no longer able to remain the idealised object that reflects back to the patient their idealised self. Under the weight of constant demand that their ‘admiration-worthy’ self-image remains untarnished at all times, the patient experiences any situation that feels contrary to this demand as an attack.

Miya too seems to have experienced intolerable distress from the perceived failure to maintain her self-image, and the idealised relationship became the source of attack. For a person whose defences, such as splitting, extreme forms of idealisation and devaluation/denigration, indicated narcissistic functioning, these mental images in the light of the emphasis on an unblemished self-image can turn severely threatening as they are experienced as enormously painful. *“Everyone has images of himself, the world, and those around him, and behaves*

as though those images, rather than the objects they represent, were the truth.”

(Berne, 1971, p. 45)

While elaborating his views on narcissism, Freud speaks of one's inability to reach out to the world as long as one is suffering from pain (1914). In extreme pain from her unconscious belief that she had betrayed her self-image and lost the idealised relationship, the only way Miya could cope was by rushing to safeguard herself from fragmentation and moving into a narcissistic way of relating, thereby losing contact with me.

Now I share a brief description of my experience with another patient who used narcissistic defences to deal with unforeseen situations threatening self-image.

Lisha was a middle-aged woman who took pride in her ability to be kind to people; a way to feel special to them and a way to share a relationship that felt more special than others. Everything would be fine until the other person would say or do something that was experienced by Lisha as a dismissal of her. For a few years Lisha did not encounter another patient in my clinic as I had online sessions before and after hers. This changed when I began meeting a patient in person just before Lisha's time.

On a day when Lisha arrived early to the session, she noticed the other patient leaving my clinic. As she lay down on the couch, she turned to me and asked if I was well. On exploring, I understood that seeing the other patient had triggered a recognition that we were not a special unit, that she was not special

as my only patient. When Lisha became aware of that, she was exposed to her extreme sense of inadequacy, which was split-off and projected onto me. She became frightened that I was now an incapable object that would be unable to provide her with the devotion that she sought. Lisha had to render me unwell in her mind so that she could reinstate the narcissistic relationship, revive me and establish us as special again, as a way to bring relief to herself.

Patients like Miya and Lisha are threatened by a pain that forces them to retreat from contact with me. The maintenance of the narcissistic relationship is crucial for such patients because, when they feel one with the therapist, they feel seen and protected, and when they feel forcibly pushed into separation by an action of the therapist, they feel humiliated. We can extend this understanding to every relationship the person has.

“Seeing and being seen are important aspects of narcissism, where self-consciousness is always a feature, and one which becomes acute when a patient loses the protection of a narcissistic relationship and is obliged to tolerate a degree of separateness.” (Steiner, 2006, p. 939).

The experience of oneness with the therapist gives rise to pride and being admired, which feel pleasurable, while the experience of getting severed from the therapist gives rise to unbearable feelings of extreme shame, embarrassment, and humiliation. To prevent the experience of separation and the resulting smallness from which the patient feels provoked to retaliate, the patient seeks to avoid any indication of their dependency on the therapist.

As seen with Miya and Lisha, the patient is threatened with breakdown by any awareness of this dependency, and narcissistic defences are erected immediately to bind back the fragmenting self. The patient ensures the banishment of any need for the therapist by slipping into their psychic retreat.

Escape from Reality

“A psychic retreat provides the patient with an area of relative peace and protection from strain when meaningful contact with the analyst is experienced as threatening.” (Steiner, 2004, p.1)

With a narcissistic organisation of personality, a patient is able to inhabit and maintain a psychic retreat which, while serving to mitigate anxiety at the same time cuts off meaningful contact with others and with reality.

Coming back to Miya, she had coped in every relationship by regarding herself as superior and telling herself she did not need anyone. I detected this attitude towards me as well. Miya would tell me that she was doing all the work in our sessions by herself, and my contribution barely mattered. “I am enough for myself. I am all I need.” The world she believed she deserved and belonged to, in which there was no other (her psychic retreat), would also be explicitly described by her as one in which she could replace one person with another if any conflict arose.

What transpires in the patient that they feel compelled to attack the very therapy that they consistently participate in through their dedicated efforts and

feel helped by? To shape my understanding further, I now turn to an idea proposed by Melanie Klein.

Good Feelings, and Bad Feelings

In a significant breakaway from Freud's emphasis on the energy and drive theory, Klein posited that from the beginning of life the personality of a baby develops according to the two-way interaction and response between mother and baby (Hinshelwood and Fortuna, 2018).

A baby who experiences the satisfaction of the mother/other's attunement is likely to return the happiness with smiles and joy, which motivates the mother/other to offer more. Similarly, a mother who cares for the baby's sense of bliss is more mindful of 'listening' and contributing to this baby's communication and encourages the baby to engage deeper with her.

There are moments, of course, when the mother/other is not available in the way the baby wishes for her to be. The baby would have to deal with the frustration of its needs, for example, when the baby is desperate with hunger and the bottle of milk is not yet ready. The crying baby may be overwhelming for the mother, and she may respond to the baby's distress with apathy. Just like good feelings foster good feelings, the absence of good feelings, which could be seen as represented by feelings of hate, too can get aggravated.

These feelings of goodness and badness become very important when we consider their impact on the baby's experience of itself. A predominance of good

feelings contributes to the baby feeling good about itself (I am good), and being besieged by bad feelings leaves the baby with a sense that it is bad.

“The whole thrust of her approach was to understand the balance between love and hateful feelings, and the persistent anxiety that the balance may become disturbed.” (Hinshelwood & Fortuna, 2018, p.116)

Klein, referred above, gave much significance to the capacity to experience a balance between love and hate. As the good feelings get reinstated, the balance is restored. In those circumstances where one is unable to develop a capacity for this balance, how might one cope?

Something needs to be Re-established

In the absence of the capacity to restore the balance between love and hate where feelings of hate outweighed love, Lisha and Miya experienced much pain from feeling dependent on the therapist who now became the source of the hate, and sought to manage the pain through narcissistic defences. They reestablished our relationship in patterns of coping that were familiar to them.

Lisha avoided becoming aware of dependency by reestablishing an ideal relationship through making me unwell so she could attend to me and make us special again. Miya avoided any dependency by denigrating our relationship. Both these patterns are narcissistic stances as Miya and Lisha could be immersed only in themselves in that moment.

Of course, it is not just the patient who is the human being in the room. My narcissistic defences too kicked in. I experienced pain and anticipated getting rejected by Miya and Lisha. I noticed my urge to retreat from my relationship with them as I struggled to find my balance.

Conclusion

“...each disorder is an intelligent attempt to solve an existential problem. Even though these solutions may in themselves be highly disturbing, if the clinician can grasp their specific intelligence and help the analysand to understand this, then a natural process of detoxification can begin.” (Bollas, 2021, p.9)

The solution created by Lisha and Miya helped maintain a semblance of safety even though the patterns were dysfunctional. As patients develop resources in their therapeutic journey, my hope is for these patterns to transform into meaningful ways of relating.

I conclude with a quote that offers a hope for change and invites the courage to be the change.

“One of the important things that human society depends on is that the negative vicious cycle ...has to be broken somehow, and the retreat of one of the parties from the endless negative cycling is important, which entails a deeply important moment that can reverse everything.” (Hinshelwood and Fortuna, 2018, p. 117)

Whether in the therapy room or outside, breaking out of the negative cycle to nurture fulfilling relationships is valuable to our shared existence in our world.

Acknowledgements

The author wishes to thank her supervisors, Dr Amit Kulkarni and Ms Gouri Salvi, for their generous and thoughtful support. She also wishes to acknowledge the SAJTA team for their helpful and professional involvement at every step. She expresses her gratitude to Dr Susan George, TSTA, for her encouragement through the years.

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GUIDELINES AND POLICIES

GUIDELINES FOR WRITING FOR SAJTA

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Only original material - not previously published in English and not under review by another English-language publication - is eligible for consideration and inclusion in the *SAJTA*. As an exception, any paper previously published in a language other than English will be considered if translated into English prior to submission.

If you are an author considering a submission to the *SAJTA*, the first step is to determine that your paper is well written in terms of content, structure and style. The second step is to prepare your manuscript according to the following technical requirements and then to submit it as a Word document via e-mail to the *SAJTA* Managing Editor at journal@saata.org.

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General Writing Guidelines

- Manuscripts must be submitted in Word format with a target length of 2,500 words, allowing for a flexibility range of ± 500 words (i.e., between 2,000 and 3,000 words). This word count excludes bibliographic references, keywords, and the author biography. Authors are advised to adhere strictly to this word limit, as any additions or deletions during the revision process may significantly impact the final word count. For research articles, specific word count requirements may be adjusted in accordance with the scope and nature of the submission and will be determined on a case-by-case basis.
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- Follow APA style of formatting, references, citations and bibliography (<http://www.apastyle.org/>).
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- Keep sentences short and precise. Where there are multiple ideas, construct them as two or more sentences.
- Use simple and easy-to-understand words. Remember that your audience may not be Transactional Analysis literate or even use English as their first language.
- Review the paper for redundancy, where the same idea is expressed multiple times.
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- Double-check Transactional Analysis terminologies and their definitions, e.g., names of games, components of script, drivers, injunctions etc.

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- Select a strong title, which crisply explains what the article conveys.
- Begin with an abstract / introduction, which describes what readers can expect.
- Convey the main content / discussion (methods, data, evidence, results, links), using appropriate sentences, paragraphs and apt headings / sub-headings.
- Use diagrammatic representations where appropriate, labelling figures and tables.
- Include a conclusion and list of references at the end.

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Once you have met the above submission requirements and timelines, you will receive a 'receipt of your submission' mail in a week. Over the next 3-4 weeks, your manuscript will be anonymously reviewed by at least two of *SAJTA's* co-editors, who will assess basic readability (i.e., professionally written English, clarity and novelty of the ideas presented, relevance for the transactional analysis community and its contributions to the evolution of transactional analysis theory, principles, and practice) as below.

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- Is there an adequate review of the relevant literature - Transactional Analysis as well as the literature of other related fields, such as cognitive research, neuroscience studies, organizational behavior and consultation, mother/infant research, systems theory, educational theory and practice, psychoanalytic perspectives, ethics, anthropology, social/political literature, etc.?
- The editors and reviewers also consider whether the author's writing is professional and clear:
 - Is the title both engaging and reflective of the paper's content?
 - Is the abstract accurate, concise, quickly comprehensible and informative?
 - Does the overall structure of the paper help to represent the author's ideas and facilitate the reader's understanding?
 - Does the text include sub-headings that help organize the material for readers?
 - Is the author's writing style easy to read and lively? Is it professional, for example, taking care to avoid use of the male pronoun when referring to people in general?

- If the paper includes figures, tables, or other graphic images, are they necessary, easily understood, and integrated well into the main text and with the ideas of the paper?
- Does the author end the article with a conclusion (rather than a summary), one that engages the reader in thinking about further questions the article might raise, how others might build on or further develop the ideas presented, creative uses of the material, etc.?

The editors and reviewers also check to see that the paper has been prepared with care, especially in regard to proper grammar, correct (British English) spelling, and an absence of typographical errors:

- Are quotations and publications referenced correctly in the main text of the paper, giving author name, year of publication, and page number(s)?
- Is there any plagiarism of ideas or concepts which have not been appropriately referenced?
- Does the author preserve anonymity by referring to her or his previous publications in the third person, for example, “Smith (2001) has argued...” instead of “In an earlier paper, I (Smith, 2001) argued...”?

Acceptance Of Manuscript And Publishing

Around 4 weeks after your submission, you will be notified if your manuscript has been accepted for publishing and if additional modifications are needed. If not accepted, reasons will be shared. A Co-Editor from the *SAJTA* editorial team is assigned to work with each author over the next 3-4 weeks. They will collaborate closely - to challenge ideas or thinking, question the logic or rationale or make suggestions. The work will go through a few iterations before both Author and Reviewer agree to submit it to the managing editor. The managing editor will make the final decision regarding the publication of the work. Some minor changes might occur as part of the final editorial process, prior to publication.

Once ready for publishing, you will be required to submit an electronically signed *AI Disclosure and Author Consent Form* allowing exclusive publication rights to *SAJTA* and a short 150-200-word biography of yourself, with email ID for readers to correspond with you.

We hope this is useful, as you contemplate contributing to the journal.

For queries, feedback and suggestions, write to us at journal@saata.org

GUIDELINES FOR AUTHORS - THE CURIOUS CASE OF CASELETS

SAJTA is excited to introduce The Curious Case of Caselets - a dedicated space for practitioners to share concise, insightful case studies from their practice. This initiative welcomes brief yet powerful narratives that capture real-life practice encounters, innovative interventions, or reflections on working with a client, a model, or a tool.

What is a Caselet?

A caselet is a short, focused case study (800-1500 words) that provides a glimpse into a particular aspect of practice from any field of application in TA and allied fields. It is not a full research paper but rather a bite-sized, practice-oriented reflection on:

- A new model or approach or a unique way of applying an existing model
- A client session that provided a breakthrough, challenge, or learning
- A tool or method (e.g., contracting, chair work, dream work, termination, or use of creative mediums like colours, music, or movement, sharing best practices, etc.)
- An exploration of cultural or systemic influences in practice
- Reflections on ethical dilemmas, supervision, or personal growth as a practitioner

Submission Guidelines

1. Word Count: 800-1500 words
2. Structure:
 - Title (Concise and engaging)
 - Context/Background (Briefly introduce the setting and key aspects)
 - The Case (Present the client interaction, model, or tool in action)
 - Key Insights & Reflections (What did you learn? What questions remain?)
 - Conclusion (Summarize key takeaways for practitioners)
3. Confidentiality: All identifying details of clients must be altered or anonymized
4. References: APA format
5. Format: Submissions should be in Word format (DOCX), 12-point font, double-spaced in line with APA
6. Review Process: Submissions will undergo SAJTA editorial review for clarity, relevance, and ethical considerations

Who Can Contribute?

- Practicing therapists, counsellors
- Educators and organizational practitioners using Transactional Analysis (TA) and allied approaches
- Supervisors and trainers

Why Submit?

- Share your insights with a diverse and engaged community
- Contribute to a growing repository of practical, real-world experiences in TA
- Contextualize TA theory and practice through the lens of culture and systems
- Inspire and learn from fellow practitioners

We look forward to your unique contributions. Please send your submissions or queries to sajta@saata.org

ETHICAL USE AND DISCLOSURE OF ARTIFICIAL INTELLIGENCE GUIDELINES

The South Asian Journal of Transactional Analysis (SAJTA) is committed to upholding the highest standards of academic integrity and originality. The emergence of generative Artificial Intelligence (AI) tools presents new opportunities and challenges for scholarly work. These guidelines establish a framework for the transparent and accountable use of AI in manuscripts submitted to SAJTA. The core principle is that authors retain full responsibility for all content in their submissions, including any portion developed with AI assistance. This principle is fundamental because AI tools lack moral agency and legal accountability. They cannot be held responsible for the accuracy, originality, or ethical integrity of the work. Only human authors can ensure the work meets scholarly standards, verify all outputs, and be held accountable to the readers and the community.

SAJTA's policy hinges on a critical distinction between prohibited "AI-Generated" content and permitted "AI-Assisted" work. SAJTA prohibits the submission of AI-generated content. Submission of AI-generated content is considered academic misconduct. This ensures AI enhances, rather than replaces, the author's intellectual contribution.

	AI-Generated Content	AI-Assisted Writing
Description	Where AI creates the substantive intellectual output – The author's role is limited to minor editing or prompting.	The author creates the substantive intellectual output and uses AI as a tool to refine, improve, or streamline their own work.
Author's Role	Passive curator or editor of AI-created material.	Active creator, thinker, and writer who uses AI for support.
Examples	Submitting text from an AI prompt as a manuscript section.	Using AI to improve the clarity and flow of fully drafted paragraphs.
	Using AI to perform core data analysis or interpretation without human intellectual leadership	Using AI to brainstorm initial ideas to critically evaluate and develop.
	Generating an article's core argument or discussion points via AI.	Using AI to check grammar or format references.
		Using AI to summarize one's own notes or transcripts.
SAJTA's Policy	Strictly prohibited – Submission of AI generated content is considered academic misconduct.	Using AI to find or verify references.
		Permitted, but with mandatory disclosure. The author must transparently report all AI assistance in the consent and AI disclosure form.

Ethical AI Usage

1. Author Accountability and Responsibility: The author owns every word and idea in the submission. Here are a few guidelines for the author to keep in mind:

- Pause and reflect; use AI tools mindfully
- Attend to the ethical values of respect, empowerment, commitment to the relationship, responsibility and protection.

Read further about them here: https://itaaworld.com/wp-content/uploads/2023/05/12-5-14-Revised-Ethics_0.pdf

2. **Verify All Content:** Authors must thoroughly review and fact-check any AI-assisted content before submission. AI outputs can be plausible but incorrect. The Authors Guild (2025) warns that “you cannot trust the accuracy of any factual information provided by generative AI”. Every reference, data point, and factual claim in the paper must be verified against original sources. Likewise, authors must be alert to potential biases in AI output (gender, racial, or cultural biases); authors should not let AI-assisted language perpetuate stereotypes.
3. **Respect for Intellectual Property:** Be mindful that AI models are often trained on copyrighted materials. If AI paraphrases or quotes a source, check for inadvertent copyright infringement. Any text that closely resembles a known source must be properly cited or reworked. In general, AI output must be treated as any literature: if it’s not the author’s original idea or phrasing, attribution must be provided.
4. **Enforcement and Compliance Review Process:** SAJTA editors and reviewers will enforce these policies. Manuscripts may be screened with AI-detection software or plagiarism checks. If undisclosed AI use is discovered, the paper may be returned for clarification or rejected. As Hosseini et al. (2025) note, banning AI is not effective - instead SAJTA opts for regulated transparency. Authors who fail to disclose significant AI assistance risk being found in violation of publication ethics.

Ethical Disclosure and Documentation requirements:

1. **Manuscript Disclosure:** Authors must include a statement in the cover email during manuscript submission specifying any AI assistance. This statement should mention the tool name, developer (e.g. OpenAI's ChatGPT) version used, and the nature of its use. For example: "The authors (I) have used ChatGPT (OpenAI, GPT-4) to help refine the initial outline of the literature review. All AI-assisted text was subsequently verified and approved by me."
2. **Content of Disclosure:** The Author Consent and AI Disclosure form must clearly state which AI tool(s) were used and for what purpose (e.g. "used to check grammar", "used to summarize coding transcripts").
3. **Location of Disclosure:** If AI was used for data analysis, figure generation, or any other aspects of the research methodology, it should be detailed in the Methods section as one would describe other analytical tools. The author must take full responsibility for the accuracy of the analysis and must ensure that any interpretations made on the analysis are their own. SAJTA will add a footnote stating, "The author presents their original ideas in this article and acknowledges the use of AI-assistance in the refinement of this article, as disclosed to SAJTA."

Consequences

Breaches of the AI policy (e.g. fraudulent presentation of AI content) can lead to rejection of the submission, and if discovered post-publication, may result in retraction or correction. Authors should treat AI disclosure like any other ethical requirement (similar to conflict of interest or funding disclosures).

SAJTA will review these guidelines regularly to adapt to the evolving technological landscape. Authors are expected to comply with the latest version published on the SAJTA website.

References

Ai best practices for authors. The Authors Guild. (2025, May 22).

<https://authorsguild.org/resource/ai-best-practices-for-authors/>

Resnik, D. B., & Hosseini, M. (2025). Disclosing artificial intelligence use in scientific research and publication: When should disclosure be mandatory, optional, or unnecessary? *Accountability in research*, 1-13.